

Jaundice

» What causes jaundice in babies?

Jaundice in newborns, known as neonatal jaundice, is a usually harmless condition caused by high levels in the blood of the substance bilirubin. Bilirubin is a yellow pigment that is produced when old red blood cells are broken down and metabolized by the liver, then excreted in the urine and feces. In the uterus, your baby needed extra red blood cells to carry oxygen to her own body and the placenta. However, for life outside the uterus she needs fewer red blood cells, and has to break down the excess. At birth, the extra red blood cells mean an extra load on your baby's immature liver; the liver can't keep up and therefore bilirubin is reabsorbed back into your baby's blood stream, causing yellowing in the skin and the whites of the eyes. The condition is very common in newborn babies, affecting 6 out of 10 babies born at term, and 8 out of 10 born prematurely. Premature babies are slightly more likely to be jaundiced since their immature liver and gut are even less well equipped to deal with an overload of red blood cells. Jaundice usually arises in the first two to

three days of life, and in most babies resolves itself without treatment within a week or so. In a few babies, jaundice becomes severe, and the babies require treatment to avoid serious, but rare, complications affecting the brain.

» How will I know if my baby has jaundice?

If you notice a yellow tinge to your baby's skin, press the middle of her forehead with your finger (the skin yellowing progresses from top to toe): if your baby has jaundice, the color of the depressed skin will be yellow when you remove your finger. You may also notice a yellowing on the palms of the hands or soles of her feet. Stools may be pale (rather than brown), and urine very yellow (rather than pale). The whites of your baby's eyes may appear yellow, too.

» Can jaundice be missed?

You may not notice mild jaundice, but the pediatricians and nurses at the hospital will check your baby regularly for jaundice, and when you go home, your pediatrician will check her again within a couple of days, so it's unlikely that it will go unnoticed. Your doctor will also take into account risk

factors for jaundice, such as whether or not your baby is premature, if you have other children who had neonatal jaundice, whether your baby is feeding well, whether you're breast- or bottle feeding (it is more common in breast-fed babies), and whether your baby seems alert and responsive. Poor feeding and an uninterested baby can make a case for likely jaundice. Call your doctor immediately if yellowing skin is accompanied by extreme lethargy, irritability, unwillingness to feed, or a fever.

» Will my baby have to have any tests to confirm suspected jaundice?

If the hospital is concerned about the level of your baby's jaundice, they will use a special light that shines through the skin to show bilirubin. Your doctor may also take a small blood sample to test for bilirubin levels in your baby's blood, and he or she may take a urine sample to check for an underlying condition. If jaundice is confirmed, doctors will do further tests on your baby's blood to try to establish the cause and severity.

» Will my baby need to be treated?

If jaundice is mild, your baby will usually be left alone to allow the liver to mature and resolve the jaundice naturally—usually the liver is able to metabolize excess bilirubin by two weeks after birth. If your baby is premature, or she has very high levels of bilirubin, she will be treated with a form of light therapy called phototherapy. Treatment is usually completely successful within a few days and, assuming all else is well, your baby will be able to go home. If bilirubin levels remain high, a blood transfusion may be given in a neonatal intensive care unit. Small amounts of your baby's blood are replaced with matching donor blood until the levels of bilirubin are low enough that the jaundice dissipates.

» Is there anything I can do to prevent my newborn from getting jaundice?

Make sure that your baby feeds well so she doesn't become dehydrated. If you're breast-feeding, encourage her to feed every two hours. If you're bottle-feeding, try for a 30 to 60 ml feed every three hours. If you're breast-feeding, but your baby isn't taking in enough milk and begins to show early signs of jaundice, your doctor may suggest you use a breast pump to express your milk for a day or so (which will also encourage your milk-flow), feeding it to your baby in a bottle.

Phototherapy treatment Your baby is placed under a special light that changes the chemical makeup of bilirubin to make it easier to break down.

